جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : انهام محمد ابراهيم محمد
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** ///
رقم البطاقة : 24812031401262 **رقم التليفون :** 01127439204 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 03-12-1948 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: انهام محمد ابراهيم محمد
National ID: 24812031401262

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

MDT FORM

Patient Name	أتهام محمد إبراهيم	Age	76
Sex	F	Special habits	
Family History		Comorbidity	HTN
Surgical History		Telephone	01127439204
Menopausal Status	Post	Marital Status	Widow
ID	24812031401262	Medical ID	1937/2024
Date	7/10/2025	Reason for MDT	PD

Medical Report

Pathology: IDC + ILC

ER 8/8 PR 6/8 HER2 0 Ki67 15%

T2 N1M1

- Presentation :
- History : underwent simple mastectomy + sentinel LN ⅗ + ECE
  FOR PORT + adj AI
  12/24 BONE SCAN: SUSPICIOUS UPTAKE

  1/2025 multiple porta hepatitis , para aortic . rt iliac

  3/2025 for fasoldex + zometa /3 m – patient received 5 cycles of faslodex

  4/25 post contrast CT CAP – PD of right iliac bony lesion 3.5*3 cm vs 2.3*2 cm

  9/25 bone scan: active lesions left 7th rib, LV3

  Right acetabulum, femoral head
- Recommendations :

MDT DECISION	For A Romasine / Palbociclib +zometa / 3 m

Name	د/ خالد عبد العزيز	د/ هاشم عبد الرحمن	د/ ابو بكر حجازي
Signature			

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Patient's Name: Enham mohamed ibrahim
Date: 2026-03-24
Referred by: Prof.Dr. cap+c
Hospital: **Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Status follow up, in reviewing the previous limited study report dated 28-1-2025

I- CHEST:

- Left sided mastectomy with no operative bed gross enchanting lesions.
- Still noted reduced left hemithorax with ipsilateral mediastinal shift.
- Left pleural thickening with rim of thick calcifications
- Left multiple fibrotic bands.
- Still noted right lung compensatory hyperinflation and retrosternal herniation.
- Right lower lobe air filled cyst.
- no sizable pulmonary nodules, consolidation or cavitation.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No plural or pericardial collection.
- No gross cardiomegaly

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Normal C.T appearance of the spleen, pancreas, both kidneys, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No pathologically enlarged abdominal or pelvic lymphadenopathy.
- No ascites.

III- Skeletal survey:

- Right hip osteoarthritic changes with narrowing of joint spaces, marginal ostophytic lipping and subchondral cystic changes
- Dorsolumbar scoliosis.
- Dorsolumbar spondylodegenerative changes

Opinion:

- **Status follow up showing rather stationary course since last study.**

Dr.Nourhan Ewis

Dr.Nourhan Ewis

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE
SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : inham Mohammed Ibrahim Mohammed
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 14-03-2026 08:16
Medical No. : 332036
Ins.Company : تعاقد مستشفيات

Tc99m MDP SKELETAL SCINTIGRAPHY

History:

- 77-year-old female with history of metastatic breast carcinoma, presenting with bone aches. Follow-up assessment of known osseous metastatic disease.
- **Prior bone scan (05/09/2025):** Scattered osseous lesions at left 7th rib, LV3 and right acetabulum/femoral head.

Technique:

- Anterior and posterior whole-body scans were acquired 2-3 hours post injection of the tracer.

Findings:

- Persistent radiotracer uptake within the old osseous lesions involving the left 7th rib, LV3 and right acetabulum/femoral head. When compared to the prior study dated 05/09/2025, these lesions demonstrate a **stationary course**, with no significant interval change in size or intensity of tracer uptake.
- No new foci of abnormal radiotracer uptake are identified to suggest progressive metastatic disease.
- Age-related degenerative/arthritic changes in the sternum, both shoulders, and multiple weight-bearing joints.

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE BONE_SCAN]]
Call Center 19656

Name : inham Mohammed Ibrahim Mohammed
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt
Date : 14-03-2026 08:16
Medical No. : 332036
Ins.Company : تعاقد مستشفيات

Impression:

- **Stable osseous metastatic disease**; known metastatic lesions in remain **stable with no new lesions identified**. This appearance suggests a favorable response to ongoing therapy.
- Clinical correlation and continued follow-up per oncology protocol are recommended.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Patient Name	: الأستاذه / انهام محمد ابراهيم محمد صالح	Request No.	: 13002424
Patient No.	: 24812031401262	Request Date	: 30/05/2024
Path No.	: 1819/24	Verification Date	: 22/06/2024
Sex	: Female	Financial Category	: اورام دار السلام - مباشره
Age	: 75 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed yousef

Pathology Report

Clinical Data
Left breast mass

Nature of specimen
Tru cut biopsy, collected at 30/5/2024, 10:30 am

Gross examination
Multiple grayish white tissue cores up to 2 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
* Histological type: Invasive duct &lobulr carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
* LCIS: Positive
* Angioinvasion: Couldn't be detected in the received biopsy
*Perineural invasion: Positive
* Microcalcification: Positive
• IPT (Positive control showed appropriate reactivity) :
-E-cadherin:Focally positive
* ER: Strong nuclear reaction in 80 % tumor cells (Alired score 8/8)
* PR: Strong nuclear reaction in 25 % tumor cells (Allred score 6/8)
* HER2: No membrane reaction in of tumor cells (score 0)
* Ki 67: 15 % nuclear reaction of tumor cells

Verified by :
Dr.Heba Hasan

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / انهام محمد ابراهيم محمد صالح	Request No.	: 13002424
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Age	: 75 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Diagnosis
Left breast mass, Tru cut biopsy:
Invasive duct & lobular carcinoma, Grade II
IPT:

* E-Cadherin:Focally positive
* ER: Positive (score 8/8).
* PR: Positive (score 6/8).
* HER2: Negative (score 0).
* Ki67: Positive in 15% of tumor cells.

Signature
Prof. Dr. Lobna Omar
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذة / انهام محمد ابراهيم محمد صالح	Request No.	: 13001426
Patient No.	: 24812031401262	Request Date	: 14/08/2024
Path No.	: 2918/24	Verification Date	: 25/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - ميكدره
Age	: 75 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Up-Front.
Nature Specimen: Left simple mastectomy and SLNs.

Gross Examination:
* Procedure: Left simple mastectomy specimen measured 22x16x5 cm with covering skin ellipse measured 23x13 cm.
* Tumor extent: A greyish white mass.
* Size: 4x3x2cm
* Skin involvement: Unremarkable grossly.
* Distance from margins: 0.4 cm away from deep margin.
* Lymph nodes number and size: Fibrofatty tissue pelce measured 4x4 cm entangling 5 LNs (4 frozen and I left for paraffin).
* Also detected 3 LNs attached laterally up to 1.5x1.5 cm.

Microscopic Examination:
* Histological type: Mixed invasive ductal and lobular carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade (3+2+1) II.
* Angioinvasion: Positive.
* Ductal carcinoma in situ and lobular carcinoma in situ: Positive (40%).
* Skin involvement: Free nipple and areola.
* SLNs: Positive in (1/4), paraffine LNs (0/1) + (1/3).
Total nodal statue: Positive in (3/8) with extracapsular extension.
* Resection Margins: Free margins.
* Frozen section results:
Positive SLNs (1/4).

Diagnosis:
Left breast, simple mastectomy and SLNs Biopsy:

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الانشلاء / الهام محمد ابراهيم محمد صالح	Request No.	: 13001426
Patient No.	: 24812031401262	Request Date	: 14/08/2024
Path No.	: 2918/24	Verification Date	: 25/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - مباشره
Age	: 75 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Mixed invasive ductal and lobular carcinoma, Grade II.
Free deep margin.
Free nipple and areola.
SLNs: Positive in (1/4), paraffine LNs (0/1) + (1/3).
Total nodal statue: Positive in (3/8) with extracapsular extension.
Stage: pT2, pN1a.

Signature
Dr. Salma Tarek.
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Anham Mohamed Ibrahim Mohamed Saleh	Date : 13/07/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 75 Year(s)
Medical ID: 1937/2024	Pat. ID: 64982

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows retroareolar spiculated irregular mass lesion, of high density.
- Complementary ultrasound revealed left breast irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, 12-1 o'clock nape-areol r o'clork position w2ith intraductal extension toward the nipple, 0.3cm from the overlying skin, measuring 3.5X2cm, consistent to the palpable lump,
- Normal skin thickness, and nipple-areola complex of the right breast.
- Single prominent left axillary level I lymph node measuring 1.3X0.7 cm with muffled hilum.
- No left infra or supraclavicular lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast UOQ malignant looking mass lesion (BIRADS 6)
- I8ndetermiante left axillary LNs.
- Benign findings right breast (BIRADS 2)

Yours truly

Dr.Yomna tarek,M.Sc.

Not Approved

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name : Elham Mohamed Ibrahim Mohamed
Branch : Dokki

Date : 05-09-2025 02:15
Medical No. : 195085

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- 76-year-old lady, with history of left breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- Enhanced tracer uptake seen at left 7th rib, LV3 and right acetabulum / femoral head.
- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen allover the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- Few active skeletal lesions at the aforementioned sites; for more radiological correlation.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name: Anham Mohamed Ibrahim Mohamed Saleh	Date : 09/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 77 Year(s)
Medical ID: 1937/2024	Pat. ID: 64982

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

History: Known case of left breast cancer.

Findings: (compared to previous study dated 28-01-2025.)

I-Chest

- Left interval MRM with operative bed showing no sizable cysts, collections or nodules.
- Still noted:
  -both lung lobes subpleural reticulations with left sided septal thickening
  -Encapsulated left pleural effusion with unilateral calcified wall, restricted fibrotic lesions and reduced left lung volume with shift of mediastinum and compensatory right lung hyperinflation.
- Regressive course regarding previously noted few prominent mediastinal lymph nodes; largest is seen pre-carinal measuring 9 x 5mm compared to 8 x 13 mm.
- Few sub millimetric bilateral supraclavicular lymph nodes.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No right pleural effusion.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:(compared to previous study dated 23-06-2024.)

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Small left renal cortical cyst is noted measuring about 1 cm.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Multiple prominent porta-hepatis, para-aortic, mesenteric, and right iliac lymph nodes, largest is seen para-aortic measuring 1.1 x 0.9 cm.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Still noted right iliac bone lytic lesion with bubbly appearance is seen with progression measures about 3.5 x3 compared to 2.3x 2 cm.
- Small sclerotic lesion of D8.

CT Scan
[[DOCTYPE:CT_SCAN]]

shaped spine with advanced sorsolumbar spondylosis.

Opinion:

- **Status post MRM showing no sizable operative bed cysts, collections or nodules.**
- **Progression of bony lesions as described .**
- **Mild regression of size mediastinal L. ns .**
- **Rather stationary course since the last studies.**

DR Reham Khedr , M.D.
Signature Reeham.Khader
11/09/2025 3:50 PM

Bone Scan
[[DOCTYPE:BONE_SCAN]]

[[DOCTYPE:BONE_SCAN]]
Bone Scan

SCAN & LABS

مركز النيل

Name : inham Mohammed Ibrahim Mohammed
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 14-03-2026 08:16
Medical No. : 332036
Ins.Company : تعاقد مستشفيات

Tc99m MDP SKELETAL SCINTIGRAPHY

History:

- 77-year-old female with history of metastatic breast carcinoma, presenting with bone aches. Follow-up assessment of known osseous metastatic disease.
- **Prior bone scan (05/09/2025):** Scattered osseous lesions at left 7th rib, LV3 and right acetabulum/femoral head.

Technique:

- Anterior and posterior whole-body scans were acquired 2-3 hours post injection of the tracer.

Findings:

- Persistent radiotracer uptake within the old osseous lesions involving the left 7th rib, LV3 and right acetabulum/femoral head. When compared to the prior study dated 05/09/2025, these lesions demonstrate a **stationary course**, with no significant interval change in size or intensity of tracer uptake.
- No new foci of abnormal radiotracer uptake are identified to suggest progressive metastatic disease.
- Age-related degenerative/arthritic changes in the sternum, both shoulders, and multiple weight-bearing joints.

Call Center

[[DOCTYPE:CT_SCAN]]
CT Scan

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient's Name: Enham mohamed ibrahim
Date: 2026-03-24
Referred by: Prof.Dr. cap+c
Hospital: **Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Status follow up, in reviewing the previous limited study report dated 28-1-2025

I- CHEST:

- Left sided mastectomy with no operative bed gross enchanting lesions.
- Still noted reduced left hemithorax with ipsilateral mediastinal shift.
- Left pleural thickening with rim of thick calcifications
- Left multiple fibrotic bands.
- Still noted right lung compensatory hyperinflation and retrosternal herniation.
- Right lower lobe air filled cyst.
- no sizable pulmonary nodules, consolidation or cavitation.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No plural or pericardial collection.
- No gross cardiomegaly

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Normal C.T appearance of the spleen, pancreas, both kidneys, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No pathologically enlarged abdominal or pelvic lymphadenopathy.
- No ascites.

III- Skeletal survey:

- Right hip osteoarthritic changes with narrowing of joint spaces, marginal ostophytic lipping and subchondral cystic changes
- Dorsolumbar scoliosis.
- Dorsolumbar spondylodegenerative changes

Opinion:

- **Status follow up showing rather stationary course since last study.**

Dr.Nourhan Ewis

Dr.Nourhan Ewis

SCAN & LABS

مركز النيل

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Call Center

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

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Date:	2026-03-24
Referred by:	Prof.Dr. cap+c
Hospital:	**Goustave Roussey International Egypt**

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Opinion:

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Dr.Nourhan Ewis

Dr.Nourhan Ewis

[[DOCTYPE:BONE_SCAN]]
Bone Scan

Name : inham Mohammed Ibrahim Mohammed	Date : 14-03-2026 08:16
Branch : Dokki	Medical No. : 332036
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستشفيات

Tc99m MDP SKELETAL SCINTIGRAPHY

History:

- 77-year-old female with history of metastatic breast carcinoma, presenting with bone aches. Follow-up assessment of known osseous metastatic disease.
- **Prior bone scan (05/09/2025):** Scattered osseous lesions at left 7th rib, LV3 and right acetabulum/femoral head.

Technique:

- Anterior and posterior whole-body scans were acquired 2-3 hours post injection of the tracer.

Findings:

- Persistent radiotracer uptake within the old osseous lesions involving the left 7th rib, LV3 and right acetabulum/femoral head. When compared to the prior study dated 05/09/2025, these lesions demonstrate a **stationary course**, with no significant interval change in size or intensity of tracer uptake.
- No new foci of abnormal radiotracer uptake are identified to suggest progressive metastatic disease.
- Age-related degenerative/arthritic changes in the sternum, both shoulders, and multiple weight-bearing joints.

NILE DIAGNOSTIC CENTER

[[DOCTYPE:BONE_SCAN]]
Bone Scan

Name : inham Mohammed Ibrahim Mohammed
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 14-03-2026 08:16
Medical No. : 332036
Ins.Company : تعاقد مستشفيات

Impression:

- **Stable osseous metastatic disease**; known metastatic lesions in remain **stable with no new lesions identified**. This appearance suggests a favorable response to ongoing therapy.
- Clinical correlation and continued follow-up per oncology protocol are recommended.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER

Name : Enham Mohammed Ibrahim Mohammed Date : 03-10-2026 01:42
Branch : Dokki Medical No. : 475504
Ref.Physician Prof : Goustav Roussey Egypt

NILE DIAGNOSTIC CENTER

Tc-99m MDP SKELETAL SCINTIGRAPHY

History:
78-year-old female patient with history of surgically treated left breast cancer, received hormonal treatment, complains of bone pain and referred for skeletal survey.
Technique: Anterior and posterior whole-body scans 2 hours post injection of 20 mCi of the tracer.
Findings :

- The surveyed skeleton revealed multiple randomly distributed foci of variable tracer uptake keeping with bone secondaries. They are located at the high parietal and frontal regions of skull bones, around both shoulders, dorso-lumbar vertebrae, anterior end of left ? 8th rib, head, nek and trochanteric region of both femora.

IMPRESSION

- Multiple active osseous lesions at the aforementioned sites most likely metastatic.
- Follow up bone scan to monitor response to therapy.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Name : inham Mohammed Ibrahim Mohammed | Date : 14-03-2026 08:16
Branch : Dokki | Medical No. : 332036
Ref.Physician Prof : Goustav Roussey Egypt | Ins.Company : تعاقد مستشفيات

Impression:

- **Stable osseous metastatic disease**; known metastatic lesions in remain **stable with no new lesions identified**. This appearance suggests a favorable response to ongoing therapy.
- Clinical correlation and continued follow-up per oncology protocol are recommended.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Medical No. :	10495
Patient's Name:	Inham Mohamed Ibrahim Mohamed
Date:	2026-09-29
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

MULTI-SLICE POST CONTRAST AXIAL CT SCAN OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Chest findings

-Known case of left breast cancer underwent MRM with bony metastatic deposits.

- Reduced left lung volume with ipsilateral mediastinal shift.
- Minimal left sided encysted pleural effusion with coarse pleural calcification...Suggestive of chronic inflammatory process
- Enlarged cardiac size, NO pericardial effusion.
- Areas of mosaic lung attenuation with basal atelectatic bands and fissural based small nodules.
- NO detected suspicious pulmonary nodules or enhanced masses.
- Clear right **pleural surface** with no evidence of pleural effusion.
- Normal **cardiac size**, no pericardial effusion.

Abdomen and pelvis findings

- Mild enlarged **liver**. No abnormal attenuation values or enhancing lesions.
- No dilated intra-hepatic **biliary radicles**.
- Normal caliber patent **Portal Vein**
- Normal **gallbladder**. No hyperdense gall stones. Normal extrahepatic bile ducts.
- Normal size, shape and parenchymal density of the **pancreas**. No enhancing lesions.
- Normal size, shape and density of the **spleen**. No focal lesions.
- Normal size, and contrast excretion by **both kidneys**. Normal parenchymal thickness. No renal, masses stones or back pressure changes.
- Bilateral supra-**renal glands thickening**.
- No **ascites**
- Normal **aorta, I.V.C.**
- Normal filling of the **urinary bladder** with smooth inner surface and uniform wall thickness.
- **Multiple variable sized sclerotic bony lesions seen scattered in the vertebral column, the largest D8 vertebra...Metastatic.**
- **Degenerative changes of the scanned bone with S shaped dorsolumbar scoliosis.**

CONCLUSION:

- **Known case of surgically managed breast cancer with bony metastatic deposits.**
- **Other findings as described.**

CT Scan
[[DOCTYPE:CT_SCAN]]